National Cancer Control Programme (NCCP) 00569 · clinician draft

Pembrolizumab + pemetrexed + cisplatin

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Induction: cisplatin-containing cycles 1–4

Active medicines: pembrolizumab + pemetrexed + cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before treatment Day 1, then daily Days 2–3
Ondansetron	16 mg orally	Day 1 Individualise for QT risk and patient factors.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	National Cancer Control Programme (NCCP) NCIS V8 high-risk general row Reconcile with pemetrexed rash steroid and aprepitant interaction; issue one clear prescription, do not add overlapping courses.
Olanzapine	5 mg orally daily	Days 1–4 Cisplatin high-risk general HEC row. Sedation/metabolic cautions; consultant may individualise.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS table does not specify an end day here; prescribe duration and suitability. Concomitant olanzapine increases extrapyramidal adverse-effect risk.
Folic acid	350–1000 micrograms orally daily	At least five doses in seven days before first pemetrexed; throughout treatment and 21 days after the final dose
Vitamin B12 (hydroxocobalamin)	1000 micrograms intramuscular (IM)	Week before first pemetrexed and every three cycles thereafter Later doses may be on the pemetrexed day.
Pemetrexed rash prophylaxis: dexamethasone equivalent	4 mg orally twice daily	Day before, day of and day after pemetrexed This is the regimen rash schedule; prescriber/pharmacy must reconcile it with antiemetic dexamethasone into one unambiguous prescription, including aprepitant interaction. Do not simply add both courses.
Cisplatin hydration and electrolytes	Local individual prescription	Before and after cisplatin NCCP suggested framework: pre 1 L sodium chloride 0.9% over 60 minutes; post 1 L over 2 hours with 10 mmol magnesium sulfate and 20 mmol potassium chloride. Local policy and fluid/electrolyte status govern. Mannitol optional; routine loop diuretic is not required unless fluid overload.

Additional phase-specific support

- Cisplatin requires high-risk antiemetics and individual hydration/electrolyte review.
- Folic acid and B12 must be in place before pemetrexed and continue into maintenance.
- Combine rash and antiemetic dexamethasone directions into one reconciled prescription; do not stack courses.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00569 v5 · pNCCP00569 pages 1–2,6; NCIS V8 Table 1 · NCCP NCIS Medical Oncology antiemetics V8

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Maintenance: pembrolizumab + pemetrexed

Active medicines: pembrolizumab + pemetrexed

Emetogenic risk: low

Medicine	Dose / route	When
Folic acid	350–1000 micrograms orally daily	At least five doses in seven days before first pemetrexed; throughout treatment and 21 days after the final dose

Medicine	Dose / route	When
Vitamin B12 (hydroxocobalamin)	1000 micrograms intramuscular (IM)	Week before first pemetrexed and every three cycles thereafter Later doses may be on the pemetrexed day.
Pemetrexed rash prophylaxis: dexamethasone equivalent	4 mg orally twice daily	Day before, day of and day after pemetrexed This is the regimen rash schedule; prescriber/pharmacy must reconcile it with antiemetic dexamethasone into one unambiguous prescription, including aprepitant interaction. Do not simply add both courses.
Low-risk antiemetic / rescue plan	Individual prescription	Around pemetrexed Account for the existing rash-prophylaxis steroid. Do not automatically continue induction aprepitant/olanzapine/high-risk steroid course.

Additional phase-specific support

- Continue daily folate, B12 every three cycles and pemetrexed rash prophylaxis.
- Cisplatin and its hydration protocol end after induction; reassess supportive prescriptions.
- Keep immune, renal, glucose and thyroid monitoring.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00569 v5 · pNCCP 00569 treatment and supportive-care sections; NCIS V8 · NCCP NCIS Medical Oncology antiemetics V8

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